

Rajesh Kumar Gupta vs Smt. Mohani Gupta on 21 February, 2024

Author: Sheel Nagu

Bench: Sheel Nagu

1

IN THE HIGH COURT OF MADHYA PRADESH
AT J A B A L P U R
BEFORE
HON'BLE SHRI JUSTICE SHEEL NAGU
&
HON'BLE SHRI JUSTICE VINAY SARAF

ON THE 21st OF FEBRUARY, 2024

FIRST APPEAL No. 478 of 2014

BETWEEN: -

RAJESH KUMAR GUPTA S/O B.R. GUPTA, AGED
ABOUT 38 YEARS, OCCUPATION: CONTRACTOR
R/O BICHHIYA, TEHSIL AND DISTRICT MANDLA
(MADHYA PRADESH)

(BY SHRI SANJAY KUMAR PATEL - ADVOCATE ALONG
WITH SHRI SUSHIL KUMAR SHARMA - ADVOCATE)

AND

SMT. MOHANI GUPTA W/O RAJESH KUMAR
GUPTA, AGED ABOUT 32 YEARS, R/O IN FORNT OF
LALIPUR PETROL PUMP, VILLAGE LALIPUR, P.S.
TEHSIL AND DISTRICT MANDLA (MADHYA
PRADESH)

(BY SHRI BHUPENDRA KUMAR MISHRA - ADVOCATE)

This appeal coming on for hearing this day, Justice Vin
passed the following:

JUDGMENT

By present appeal preferred under Section 19 of Family Courts Act r/w 28 of Hindu Marriage Act, 1955, appellant/husband assailed judgment and decree dated 5.4.2014 passed by Principal Judge,

Family Court, Mandla in Civil Suit No.1-A/2014, whereby learned Family Court dismissed petition filed by appellant/husband for grant of decree of divorce. With the consent of the parties, appeal is heard finally.

2. Appellant married respondent on 11.7.2008 as per Hindu Rights and Rituals in village Lalipur, District Mandla and they were blessed with one boy on 20.04.2011, who is presently residing with wife. Hereinafter for the sake of brevity, appellant is referred as "husband" and respondent as "wife".

3. Husband filed divorce petition on 19.06.2012 u/s 13(1)(iii) of Hindu Marriage Act, 1955 (in short 'H.M. Act') upon allegation that when wife was residing with him, he observed that her behaviour was unnatural like she used to take bath for hours, standing at one place for long time, unable to cook food properly, forget to switch off gas burner, tried to jump from roof, doesn't wear clothes properly etc. and found that mental condition of wife was not sound & proper.

3.1 It is further pleaded that when her unnatural behaviour was brought to knowledge of her father, he revealed that even before marriage she was under treatment of Neuro Physician Dr. M.S. Jauhri of Jauhri Hospital, Jabalpur and she is suffering from mental disease Schizophrenia. Husband took wife to Dr. Jauhri, who advice to consult with Psychiatrist Dr. Ashutosh Kapoor and treatment was started. Later on, she was treated at Netaji Subhash Chandra Bose Medical College, Jabalpur and at Manoyog Clinic, Nagpur, but her condition was not improved. 3.2 Husband stated in petition that under these circumstance, it was not possible to live with her and, after sending wife and minor son to her parental home, petition for divorce was filed by husband on the ground of mental disorder of wife enumerated in Section 13(1)(iii) of Hindu Marriage Act. According to allegations mental disease of wife is continuous and incurable and of such a kind and to such an extent that husband cannot reasonably accept to live with wife. Husband prayed for grant of decree of divorce.

4. Respondent/wife appeared before Family Court and denied allegations of mental disorder. She pleaded in written statement that husband asked wife to bring Rs. 50,000/- from her father and when she expressed her inability to bring amount, he treated her with cruelty and when she disclosed act of cruelty to her father, a complaint was lodged by her father on 04.09.2009 at Police Station Mandla, where husband and his family members accepted their fault and assured to behave properly in future and, thereafter, they continued residing together, however after some time husband again started beat her to fulfill his demand of money. 4.1 Wife further pleaded that husband is habitual drinker and after consuming alcohol he often beat her and abuse in filthy language. Wife raised plea in written statement that secretly sleeping pills were given to her by husband and his family members, due to which her health deteriorate. Elder brother of husband is in Police department and on the strength of power of his elder brother, husband left wife and minor son in month of February, 2012 at her parental home with a demand to bring Rs. 5.00 lacs, otherwise to live with her father.

4.2 To rebut allegation of mental disorder, wife pleaded that she is educated up to M.A. (previous). She made complaint on 08.07.2012 to S.P. Mandla, but no action was taken by Police Officers under pressure of elder brother of her husband, consequently she filed a complaint before JMFC, Mandla,

which was registered u/s 498-A of IPC against her husband and family members and still pending.

5. Learned Family Court on the basis of pleadings of parties, framed issues. Husband examined himself (PW-1), Dr. A.D. Haq (PW-2), Shyam Gupta (PW-3), Dr. Ratnesh Kumar (PW-4), whereas wife examined herself (DW-1), Virendra Kumar Gupta (DW-2), Rajendra Prasad Gupta (DW-3), Sunder Lal Patel (DW-4) and Narayan Prasad Turkail (DW-5).

6. After considering oral and documentary evidence produced by parties, learned family court by impugned judgment and decree, dismissed petition filed by husband with a direction to pay maintenance @ Rs. 4000/- per month to wife and minor son.

7. Shri Sanjay Patel, learned counsel appearing on behalf of husband submits that u/s 13(1)(iii) mental disorder is enumerated as one of the grounds for divorce and husband has proved on record that wife is suffering from Schizophrenia and was under treatment even before marriage and this fact was deliberately suppressed from husband. He submits that even after marriage, wife was taken to Neuro Physician, Psychiatrist and Councilor for treatment, but there was no improvement in her condition. 7.1 He further submits that husband intimated to Police Mandla regarding mental status of his wife and filed application before Parivar Paramarsh Kendra, Mandla. Initially he took wife to local Dr. Prahalad Kshatriya and Dr. Tarun Ahirwar and thereafter to Dr. Jauhri, Dr. Kapoor, Dr. Nimbalkar and to SIMS Hospital, Nagpur. By exhibiting medical papers Ex.P/4 to P/31 husband brought on record that wife is suffering from Schizophrenia and this fact was duly proved by PW-2 Dr. A.D. Haq in his court statement. PW-3 Shyam Gupta proved that wife was suffering from disease even before marriage. PW-4 Dr. Ratnesh Psychiatrist of Victoria Hospital, Jabalpur also proved that wife is suffering from Schizophrenia. 7.2 Shri Patel points out that wife in para 22 of her cross-examination admit that after marriage on 14.07.2008 and 20.07.2008, her husband took her to Dr. Prahlad Kshatriya for treatment, however, denied that before marriage she was under treatment of Psychiatrist of Nagpur. He prays for allowing the appeal and grant of decree of divorce.

7.3 Learned counsel for appellant relied on judgment delivered by Apex Court in the matter of Vinita Saxena vs. Pankaj Pandit, AIR 2006 SC 1662, and relied on the following paras which are reproduced as under;

"32. The word "cruelty" has not been defined and it has been used in relation to human conduct or human behaviour. It is the conduct in relation to or in respect of matrimonial duties and obligations. It is a course of conduct and one which is adversely affecting the other. The cruelty may be mental or physical, intentional or unintentional. There may be cases where the conduct complained of itself is bad enough and per se unlawful or illegal. Then the impact or the injurious effect on the other spouse need not be enquired into or considered. In such cases, the cruelty will be established if the conduct itself is proved or admitted.

33. The cruelty alleged may largely depend upon the type of life the parties are accustomed to or their economic and social conditions, their culture and human values to which they attach importance. Judged by the standard of modern

civilisation in the background of the cultural heritage and traditions of our society, a young and well-educated woman like the appellant herein is not expected to endure the harassment in domestic life whether mental, physical, intentional or unintentional. Her sentiments have to be respected, her ambition and aspiration taken into account in making adjustment and her basic needs provided, though grievances arising from temperamental disharmony are irrelevant. This view was taken by the Kerala High Court in *Rajani v. Subramonian* [AIR 1990 Ker 1 : (1990) 1 DMC 561] .

37. As to what constitutes the required mental cruelty for the purposes of the said provision, will not depend upon the numerical count of such incidents or only on the continuous course of such conduct but really go by the intensity, gravity and stigmatic impact of it when meted out even once and the deleterious effect of it on the mental attitude, necessary for maintaining a conducive matrimonial home.

38. If the taunts, complaints and reproaches are of ordinary nature only, the court perhaps need consider the further question as to whether their continuance or persistence over a period of time render, what normally would, otherwise, not be so serious an act to be so injurious and painful as to make the spouse charged with them genuinely and reasonably conclude that the maintenance of matrimonial home is not possible any longer.

43. This Court in *Digvijay Sinhji v. Pratap Kumari* [(1969) 2 SCC 279 : AIR 1970 SC 137] has held as follows: (SCC p. 281, para 5) "5. A party is impotent if his or her mental or physical condition makes consummation of the marriage a practical impossibility. The condition must be one, according to the statute, which existed at the time of the marriage and continued to be so until the institution of the proceedings. In order to entitle the appellant to obtain a decree of nullity, establish that his wife, the respondent, was impotent at the time of the marriage and continued to be so until the institution of the proceedings."

8. Per contra, Shri Bhupendra Kumar Mishra, learned counsel for wife submits that husband has failed to prove mental disorder of wife and in her statement she has denied that due to illness, she is not able to perform his household duties properly. She denied that she is suffering from mental disease. Her father also denied fact of treatment of wife before marriage. Rajendra Prasad Gupta, Sunder Lal Patel and Narayan Prasad Turkail also denied that she is suffering from any disease.

8.1 He further submits that Dr. A.D. Haq never treated wife and stated only as per instructions of husband. He accepted in cross examination that his signatures are not available on Ex.P/2 and original record of Parivar Paramarsh Kendra is also not available. In para 11, DR. A.D. Haq accepted that he had no any conversation with Doctor who allegedly treated Mohni Gupta (wife) and he is not a Psychiatrist. He accepted in para 11 that presently Mohni Gupta is not looking as dis-balanced. In para 12, he accepted that in his presence, no treatment was given to Mohni Gupta. 8.2 Shyam Gupta had dispute with sister of respondent-wife and therefore, he stated in favour of husband. The fact of

dispute was admitted by him in his cross examination. He further submits that PW-4 Dr. Ratnesh Kumar in para 4 of his cross-examination has accepted that there is not a single document on record signed by him and he has not brought original file of Hospital. He has simply advised the patient for Pathology Test. In para 9 of his cross-examination, he accepted that he has not brought notes also. In para 11, he stated that he did not advice for meditation and in para 16, he accepted that he has not brought any documents with him to prove the fact that he ever treated Mohni Gupta.

8.3 Shri Bhupendra Kumar Mishra, further submits that Dr. Prahlad is Maternal and Child Health specialist and has not been examined in Court. There is nothing regarding any mental disease in rescription of Dr. Prahlad Ex.P/4 and P/5. He further submits that Ex.P/6 and P/7 are documents of eye checkup and Dr. M.S. Jauhri has not been examined in Court. Prescription Ex.P/8, P/25 and P/26 do not contain signatures of respondent and no opinion was given in these prescriptions by Dr. Jauhri that she is suffering from any such disease that she is not able to perform regular duties.

8.4 Dr. Ashutosh Kapoor has also not been examined in the Court. Ex.P/9 and P/10, Ex.P/16, P/18 to P/24, Ex.P/27 to P/29 do not contain signatures of Mohni Gupta and there is no such opinion recorded by Dr. Ashutosh Kapoor that she is suffering from any mental disorder. Similarly, Dr. Shashikant has also not been examined. Ex.P/11 to P/14 do not contain signatures of Mohni Gupta. He further submits that no Doctor has been examined from SIMS Hospital of Nagpur and Ex.P/15 also does not contain signature of Mohni Gupta and there is no opinion of Doctor that she is suffering from any mental disorder. No Doctor from Medical College, Jabalpur has been examined. Ex.P/17 does not contain signature of wife and no opinion has been recorded in Ex.P/17.

8.5 He further submits that husband himself has admitted in para 21 of his cross- examination that documents of medical treatment of wife filed by him do not contain her signatures and in para 22 he admitted that his wife is educated up to M.A. (previous). In para 23, husband accepted that date of birth of his son is 20.04.2011 and presently son is residing with mother. He admitted in para 24 that he left his wife and minor son at wife's parental house and thereafter never went to bring her back. He accepted that his wife lived for four years with him.

8.6 Shri Bhupendra Kumar Mishra, further submits that at the most documents filed on behalf of husband may be accepted to bring the fact of continuous treatment on record, but there is not a single document on record to prove that wife is suffering from mental disorder and disease is such a kind and degree that wife cannot be reasonably expected to live with husband. He submits that learned Family Court has rightly dismissed petition and appeal be dismissed.

9. Considering arguments advanced by rival parties and perused record. Learned Family Court has decided issue No.1 in favour of husband by holding that wife was under treatment of mental disease even before marriage. Issue No.2 has also been decided in favour of husband whereby Family Court held that marriage was performed by suppressing fact of mental disease of wife. Issues No.3 and 4 regarding demand of dowry and treating wife with cruelty by husband were decided in negative in favour of husband. However, issue No. 5 has been been decided against husband, whereby Family Court has held that husband is not entitle for decree of dissolution of marriage and findings of issue no. 5 are under challenge in present appeal. Wife has not filed any cross objection and, therefore, in

absence of any challenge to findings of Family Court in respect of issues No. 1 to 4, there is no requirement to reappreciate evidence in respect of issue nos. 1 to 4 and findings recorded by family court appears to be acceptable.

10. Crucial question in appeal is that whether wife is suffering from such a disease, which may be treated as mental disorder u/s 13(1)(iii) for grant of decree of divorce ?

11. Section 13(1)(iii) provides that either of spouse can apply for dissolution of marriage in case the other spouse is of unsound mind or suffering from mental disorder. It is suffice to reproduce the provision at this stage, which is infra :-

"Section 13 Divorce -

(1) Any marriage solemnized, whether before or after the commencement of this Act, may, on a petition presented by either the husband or the wife, be dissolved by a decree of divorce on the ground that the other party--

(iii) has been incurably of unsound mind, or has been suffering continuously or intermittently from mental disorder of such a kind and to such an extent that the petitioner cannot reasonably be expected to live with the respondent.
Explanation.--In this clause,--

(a) the expression mental disorder means mental illness, arrested or incomplete development of mind, psychopathic disorder or any other disorder or disability of mind and includes schizophrenia;

(b) the expression psychopathic disorder means a persistent disorder or disability of mind (whether or not including sub-

normality of intelligence) which results in abnormally aggressive or seriously irresponsible conduct on the part of the other party, and whether or not it requires or is susceptible to medical treatment; or"

12. Apex Court in Ram Narain Gupta vs. Smt. Rameshwari Gupta, (1988) 4 SCC 247 held that in order to make out ground of divorce u/s 13(1)(iii) of HM Act, it is not sufficient to establish that spouse is suffering continuously or intermittently from mental disorder, but it must further be established that it is of such kind and extent that applicant cannot reasonably be expected to live with respondent. For ready reference, the relevant paras from said judgment are reproduced hereunder:-

"20. The context in which the ideas of unsoundness of "mind"

and "mental disorder" occur in the section as grounds for dissolution of a marriage, require the assessment of the degree of the "mental disorder". Its degree must be such that the spouse seeking

relief cannot reasonably be expected to live with the other. All mental abnormalities are not recognised as grounds for grant of decree. If the mere existence of any degree of mental abnormality could justify dissolution of a marriage few marriages would, indeed, survive in law.

21. The answer to the apparently simple -- and perhaps misleading -- question as to "who is normal?" runs inevitably into philosophical thickets of the concept of mental normalcy and as involved therein, of the 'mind' itself. These concepts of "mind", "mental phenomena" etc., are more known than understood and the theories of "mind" and "mentation" do not indicate any internal consistency, let alone validity, of their basic ideas. Theories of "mind" with cognate ideas of "perception"

and "consciousness" encompass a wide range of thoughts, more ontological than epistemological. Theories of mental phenomena are diverse and include the dualist concept -- shared by Descartes and Sigmund Freud -- of the separateness of the existence of the physical or the material world as distinguished from the non-material mental world with its existence only spatially and not temporally. There is, again, the theory which stresses the neurological basis of the "mental phenomenon" by asserting the functional correlation of the neuronal arrangements of the brain with mental phenomena. The "behaviourist" tradition, on the other hand, interprets all reference to mind as "constructs" out of behaviour.

"Functionalism", however, seems to assert that mind is the logical or functional state of physical systems. But all theories seem to recognise, in varying degrees, that the psychometric control over the mind operates at a level not yet fully taught to science. When a person is oppressed by intense and seemingly insoluble moral dilemmas, or when grief of loss of dear ones etch away all the bright colours of life, or where a broken marriage brings with it the loss of emotional security, what standards of normalcy of behaviour could be formulated and applied? The arcane infallibility of science has not fully pervaded the study of the non-material dimensions of "being".

22. Speaking of the indisposition of science towards this study, a learned Author says:

".....we have inherited cultural resistance to treating the conscious mind as a biological phenomenon like any other. This goes back to Descartes in the seventeenth century. Descartes divided the world into two kinds of substances: mental substances and physical substances. Physical substances were the proper domain of science and mental substances were the property of religion. Something of an acceptance of this division exists even to the present day. So, for example, consciousness and subjectivity are often regarded as unsuitable topics for science. And this reluctance to deal with consciousness and subjectivity is part of a persistent objectifying tendency. People think science must be about objectively observable phenomena. On occasions when I have lectured to audiences of biologists and neurophysiologists, I have found many of them very reluctant to treat the mind in general and consciousness in particular as a proper domain of scientific investigation.

.....the use of the noun "mind" is dangerously inhabited by the ghosts of old philosophical theories. It is very difficult to resist the idea that the mind is a kind of a thing, or at least an arena, or at least some kind of black box in which all of these mental processes occur. [See: John Searle "Minds, Brains and Science"

1984 Reith Lectures, pp. 10 and 11]

23. Lord Wilberforce, referring to the psychological basis of physical illness said that the area of ignorance of the body-mind relation seems to expand with that of knowledge. In *McLoughlin v. O' Brian* [(1983) 1 LR 410, 418: (1982) 2 All ER 298] , the learned Lord said, though in a different context: (All ER p. 301) "Whatever is unknown about the mind-body relationship (and the area of ignorance seems to expand with that of knowledge), it is now accepted by medical science that recognisable and severe physical damage to the human body and system may be caused by the impact, through the senses, of external events on the mind. There may thus be produced what is as identifiable an illness as any that may be caused by direct physical impact. It is safe to say that this, in general terms, is understood by the ordinary man or woman who is hypothesised by the courts..."

24. But the illnesses that are called "mental" are kept distinguished from those that ail the "body" in a fundamental way. In "Philosophy and Medicine", Vol. 5 at page X the learned Editor refers to what distinguishes the two qualitatively:

"Undoubtedly, mental illness is so disvalued because it strikes at the very roots of our personhood. It visits us with uncontrollable fears, obsessions, compulsions, and anxieties....This is captured in part by the language we use in describing the mentally ill. One is an hysteric, is a neurotic, is an obsessive, is a schizophrenic, is a manic-depressive. On the other hand, one has heart disease, has cancer, has the flu, has malaria, has smallpox..." (emphasis supplied)

25. "Schizophrenia", it is true, is said to be difficult mental affliction. It is said to be insidious in its onset and has hereditary predisposing factor. It is characterised by the shallowness of emotions and is marked by a detachment from reality. In paranoid states, the victim responds even to fleeting expressions of disapproval from others by disproportionate reactions generated by hallucinations of persecution. Even well meant acts of kindness and of expression of sympathy appear to the victim as insidious traps. In its worst manifestation, this illness produces a crude wrench from reality and brings about a lowering of the higher mental functions.

26. "Schizophrenia" is described thus:

"A severe mental disorder (or group of disorders) characterised by a disintegration of the process of thinking, of contact with reality, and of emotional responsiveness. Delusions and hallucinations (especially of voices) are usual features, and the patient usually feels that his thoughts, sensations, and actions are controlled by, or shared with, others. He becomes socially withdrawn and loses energy and initiative. The main types of schizophrenia are simple, in which increasing social withdrawal and

personal ineffectiveness are the major changes; hebephrenic, which starts in adolescence or young adulthood (see hebephrenia) ; paranoid, characterised by prominent delusion ; and catatonic, with marked motor disturbances (see catatonia).

Schizophrenia commonly -- but not inevitably -- runs a progressive course. The prognosis has been improved in recent years with drugs such as phenothiazines and by vigorous psychological and social management and rehabilitation. There are strong genetic factors in the causation, and environmental stress can precipitate illness. [See Concise Medical Dictionary at p. 566 : Oxford Medical Publications, 1980] "

27. But the point to note and emphasise is that the personality disintegration that characterises this illness may be of varying degrees. Not all schizophrenics are characterised by the same intensity of the disease. F.C. Redlich and Daniel X. Freedman in "The Theory and Practice of Psychiatry" (1966 Edn.) say:

"Some schizophrenic reactions, which we call psychoses, may be relatively mild and transient; others may not interfere too seriously with many aspects of everyday living....."(p. 252) Are the characteristic remissions and relapses expressions of endogenous processes, or are they responses to psychosocial variables, or both ? Some patients recover, apparently completely, when such recovery occurs without treatment we speak of spontaneous remission. The term need not imply an independent endogenous process; it is just as likely that the spontaneous remission is a response to non-deliberate but nonetheless favourable psychosocial stimuli other than specific therapeutic activity (p. 465)s (emphasis supplied)

28. The reasoning of the High Court is that the requisite degree of the mental disorder which alone would justify dissolution of the marriage has not been established. This, it seems to us, to be not an unreasonable assessment of the situation -- strong arguments of Shri Goel to the contrary notwithstanding.

33. This medical concern against too readily reducing a human being into a functional non-entity and as a negative unit in family or society is law concern also and is reflected, at least partially, in the requirements of Section 13(1)(iii). In the last analysis, the mere branding of a person as schizophrenic will not suffice. For purposes of Section 13(1)(iii) "schizophrenia" is what schizophrenia does".

13. In the matter of Vinita Saxena (supra), Apex Court did not grant decree of divorce only on the ground that husband is suffering from schizophrenia, but decree was passed taking into consideration additional fact that parties were living separately since last 13 years and considering humane aspects decree of divorce was passed. Paras 50, 51, 52 of judgment are reproduced here as under:-

"50. The facts and circumstances of the case as well as all aspects pertain to humanity and life would give sufficient cogent reasons for us to allow the appeal and relieve the appellant from the shackles and chain of the respondent and let her live her own life, if nothing less but like a human being.

51. In our view, the orders of the courts below have resulted in grave miscarriage of justice to the appellant who has been constrained into living with a dead relationship for over 13 years. The resultant agony and injustice that has been caused to the appellant, it is a fit case for interference under Article 136 of the Constitution and reversal of findings of the courts below which have resulted in grave miscarriage of justice.

52. In the result, the civil appeal stands allowed. There will be a decree for divorce in favour of the appellant wife and against the respondent husband. The order of the trial court as affirmed by the High Court, stands set aside. There will be no order as to costs".

14. Apex Court in the matter of Kollam Chandra Sekhar vs. Kollam Padma Latha, (2014) 1 SCC 225 has considered the aspect of grant of decree on the ground that other spouse is suffering from schizophrenia. Apex Court framed question No.1 that, whether the respondent is suffering from a serious mental disorder i.e. schizophrenia or incurable unsoundness of mind, and can this be considered as a ground for divorce under Section 13(1)(iii) of the Hindu Marriage Act, 1955?

To answer this question after considering various pronouncement of Apex Court including judgment of Ram Narain (supra) and judgment of Vinita Saxena (supra), Apex Court observed as under :

"In our considered view, the contents of the report as stated by the team of doctors do not support the case of the appellant that the respondent is suffering from a serious case of schizophrenia, in order to grant the decree of divorce under Section 13(1)(iii) of the Act. The report states that the respondent, although suffering from "illness of schizophrenic type", does not show symptoms of psychotic illness at present and has responded well to the treatment from the acute phases and her symptoms are fairly under control with the medication which had been administered to her. It was further stated that if there is good compliance with treatment coupled with good social and family support, a schizophrenic patient can continue their marital relationship. In view of the aforesaid findings and reasons recorded, we have to hold that the patient is not suffering from the symptoms of schizophrenia as detailed above".

15. In view of above pronouncement, it appears that it itself is not sufficient to grant decree of divorce u/s 13(1)(iii) that spouse is suffering from schizophrenia, but it is also required to prove by cogent evidence before Court that disease is of such a kind and degree that husband cannot reasonably be expected to live with wife.

16. Section 13(1)(iii) does not make mere existence of a mental disorder of any degree sufficient in law to justify dissolution of a marriage. The contest in which the ideas of unsoundness of mind and mental disorder occur in section as ground for dissolution of a marriage, require assessment of degree of mental disorder and its degree must be such that spouse seeking relief cannot reasonably be expected to live with other. All mental abnormalities are not recognized as grounds for grant of decree. The medical concern against too readily reducing a human being into a functional nonentity and as a negative unit in family or society is law's concern also and is reflected, at least partially, in the requirements of Section 13(1)(iii). The personality disintegration that characterises schizophrenia may be of varying degrees. Not all schizophrenics are characterised by same intensity of disease. Burden of proof of existence of requisite degree of mental disorder is on the spouse basing claim on that state of facts.

17. Coming to facts of present case and considering above pronouncements and legal proposition, findings of learned Family Court recorded in respect of issue No.5 have been examined, wherein Family Court has opined that husband has failed to prove to bring on record gravity and degree of disease and could prove only the fact of long treatment of schizophrenia.

18. Learned Family Court considered the fact that wife give birth to a male child and she alone is taking care of minor son at her parental house and if she is not normal then it is not possible for her to take care of child. Family Court has further observed that wife is educated upto M.A. (previous) and, therefore, it cannot be accepted that disease is such a kind and degree that she cannot lead a normal life or she cannot procreate a child. Therefore, looking to evidence available on record, learned Family Court decided issue No.5 against appellant/husband.

19. After considering entire evidence available on record, this Court has no hesitation in accepting findings and approach of learned Family Court, which appears to be valid and practical. Though, appellant/ husband was able to prove that respondent/wife is suffering from schizophrenia, but he failed to prove that disease is of such a kind and degree, which may be accepted for dissolution of marriage. No sufficient material was produced on record by husband except prescriptions of Doctors, which do not contain any specific finding that disease is having grave consequences. Therefore, in considered opinion of this Court, judgment and decree passed by learned Family Court, whereby petition filed by husband u/s 13(1)(iii) of H.M. Act was dismissed are just, proper, legal and not perverse and do not require any interference by this Court in this appeal.

20. Consequently, appeal fails and is hereby dismissed. No order as to costs.

21. Record of Family Court be returned back along with copy of this judgment.

(SHEEL NAGU)

(VINAY SA)

JUDGE

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